BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks	Full blood count, kidney and liver tests and magnesium; kidney filtration assessment for carboplatin; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy tests. Hearing tests and an electrocardiogram (ECG) for compromised heart function are arranged when indicated.	Full blood count, kidney and liver tests and magnesium before each cycle, with a current kidney result for carboplatin calculation. Review hearing, nerves, skin, eyes, bowel symptoms and any reactions; report changes before every weekly cetuximab visit. The team may reassess kidney filtration if function changes.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Electrolytes including magnesium and calcium are checked monthly for 2 months after the final cetuximab. Continue review of unresolved skin, blood, hearing, nerve, mouth and nutritional problems. Examinations and scans follow the individually agreed cancer plan.
Treatment and appointments	Confirm the diagnosis, aim, prescription and actual calendar.	Cetuximab starts with a 400 mg/m² loading dose over 120 minutes, then 250 mg/m² over 60 minutes weekly. Chemotherapy starts at least 1 hour after cetuximab finishes. Carboplatin is infused over 30 minutes on Day 1 at a kidney-calculated target exposure of AUC 5. Fluorouracil 1,000 mg/m² is infused over 16–22 hours daily on Days 1–4, or 2,000 mg/m² over 48 hours twice. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit.	Follow the written home plan and keep pump or line appointments where applicable.	Carboplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15, for up to 6 cycles every 21 days. Review determines what happens afterwards. Your team confirms any treatment after the combination phase; continuing cetuximab needs an agreed prescription and calendar.	Record the last dose and follow-up appointments.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Mouth, weight and swallowing	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.
Cancer assessment	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00418 v7: official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Full blood count

WHEN

At baseline and before each cycle

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and before each cycle

Guide safe treatment and identify organ problems.

DPD assessment

WHEN

Before the first fluorouracil treatment unless already tested

Checks for reduced ability to break down fluorouracil; results affect treatment choice or dose. Testing cannot exclude every severe reaction. Kidney impairment can affect one blood-based test, so the team chooses the appropriate method.

Kidney filtration assessment

WHEN

Before treatment; reassessment when kidney function or the clinical situation changes

Carboplatin dose is calculated from kidney filtration. A measured test is preferred where feasible; the team chooses a valid method and reassesses significant changes.

Hearing assessment

If part of your plan

WHEN

At baseline when clinically indicated; review new hearing or balance symptoms during treatment

Investigates hearing loss or tinnitus and helps judge platinum-related risk.

Electrocardiogram (ECG)

If part of your plan

WHEN

Before treatment if heart function is compromised; additional assessment if symptoms warrant

Records the heart's electrical activity. New chest symptoms require assessment even after a reassuring earlier test.

Magnesium and other electrolytes

WHEN

Magnesium at baseline and before each cycle; magnesium, calcium and electrolytes monthly for 2 months after final cetuximab

Detects mineral changes caused by cetuximab; low levels may cause cramps, weakness or heart-rhythm problems.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Cetuximab + carboplatin + fluorouracil · 21-day cycle: treatment and checks

Locally advanced or metastatic head and neck squamous cell carcinoma when cisplatin is contraindicated or not tolerated.

Carboplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15, for up to 6 cycles every 21 days. Review determines what happens afterwards. Your team confirms any treatment after the combination phase; continuing cetuximab needs an agreed prescription and calendar.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests and magnesium; kidney filtration assessment for carboplatin; DPD testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy tests. Hearing tests and an ECG for compromised heart function are arranged when indicated.	Full blood count, kidney and liver tests and magnesium before each cycle, with a current kidney result for carboplatin calculation. Review hearing, nerves, skin, eyes, bowel symptoms and any reactions; report changes before every weekly cetuximab visit. The team may reassess kidney filtration if function changes.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	Electrolytes including magnesium and calcium are checked monthly for 2 months after the final cetuximab. Continue review of unresolved skin, blood, hearing, nerve, mouth and nutritional problems. Examinations and scans follow the individually agreed cancer plan.
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What each check is for

1 Combination treatment

Follow your agreed treatment calendar

Cetuximab with carboplatin and fluorouracil is used to treat squamous head and neck cancer when cisplatin is unsuitable. Your team explains the treatment aim, possible benefit and alternatives for your situation.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

2 Days 8 and 15 within the same combination cycle

Follow your agreed treatment calendar

Cetuximab with carboplatin and fluorouracil is used to treat squamous head and neck cancer when cisplatin is unsuitable. Your team explains the treatment aim, possible benefit and alternatives for your situation.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.47